Capecitabine + temozolomide (capecitabine and temozolomide (CAPTEM))

Two oral chemotherapy medicines to help control advanced pancreatic neuroendocrine tumour (NET).

Locally advanced or metastatic pancreatic neuroendocrine tumour under NCCP 00505a. · NCCP 00505 v5 · Document revised 5 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Pancreatic NET CAPTEM only. Six planned cycles are reviewed individually; this is not a brain-tumour chemoradiation schedule.

My overall treatment

28-day cycle: capecitabine twice daily D1–14; temozolomide once daily D10–14; neither medicine D15–28.



Six planned cycles

The team reviews response and tolerance; holds or stopping may change the course.

A typical 28-day cycle

Day 1–9	Day 10–14	Day 15–28
Capecitabine only	Both medicines; different food instructions	Planned rest from both anticancer medicines

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Bloods / dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) / hepatitis B** Required by regimen	Counts, kidneys/liver; DPD unless already tested; hepatitis B screen.
Each cycle	**Safety bloods** Required by regimen	Counts, kidneys/liver before each cycle; liver tests also after each cycle.
As needed / after	**international normalised ratio (blood-clotting test) (INR) / response / recovery** Required / individual plan	INR with warfarin if indicated. Scans and hormone tests on your plan; liver effects may be delayed.

Between treatments

- D10–14 needs the separate anti-sickness plan. Rest days still carry infection and other risks.
- Stop capecitabine and call for significant diarrhoea, mouth or hand-foot symptoms; urgent advice for fever or sudden illness.

Capecitabine with food or within 30 minutes after; temozolomide fasting. Swallow whole. Never double or replace a vomited dose.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Energy / headache

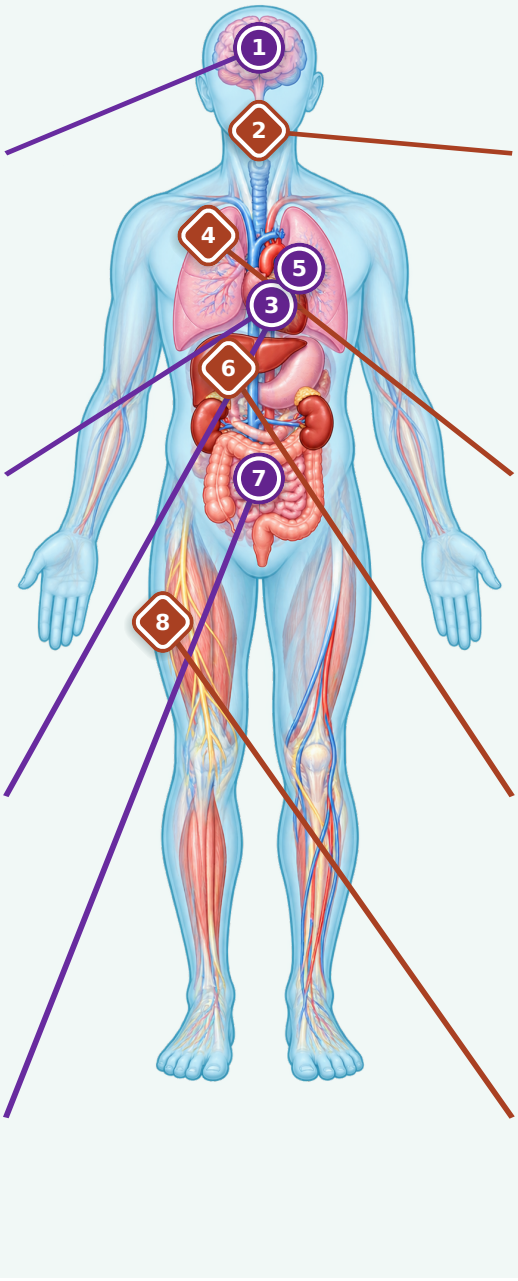
CAPTEM

Fatigue or headache can affect daily tasks. Report marked new symptoms; sudden confusion, collapse or a seizure needs emergency help.

2 Mouth / eating

Capecitabine

Mouth pain or ulcers can prevent eating and drinking. Stop capecitabine and contact the team if symptoms are significant.



3 Heart / chest

Capecitabine

Chest pain, pressure or severe breathlessness needs emergency assessment. Tell staff you are taking capecitabine.

4 Blood / infection

CAPTEM

Counts can fall, including after capsule days finish. Fever, shivering, sudden illness, unusual bruising or bleeding needs urgent advice.

5 Lungs / infection

Temozolomide

New cough, breathlessness or fever needs prompt review. The team considers infection, including PJP, and any prevention plan.

6 Liver

CAPTEM

Liver injury can develop during or after treatment. Attend tests after each cycle and report yellow skin/eyes or dark urine.

7 Bowel / nausea / fluids

CAPTEM

Diarrhoea or vomiting can cause dehydration. Take prescribed anti-sickness medicine and call for persistent symptoms or reduced urine.

8 Hands / feet / skin

Capecitabine

Painful redness, swelling or blisters can limit walking or hand use. Stop capecitabine and call when symptoms are significant.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Stop capecitabine and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, vomiting that prevents drinking, fever or sudden illness. Call urgently for new cough/breathlessness, bleeding or yellow skin/eyes. Chest pain or severe breathlessness needs emergency help. Do not restart held treatment without instructions.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

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Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

CAPTEM · pancreatic NET

28 days: capecitabine D1–14; temozolomide D10–14; rest D15–28. Six planned cycles.

Phase: _____ Planned cycles: 6

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

CUT outer border

BACK / TREATMENT RECORD

My cycle record

Team stamps after each completed cycle.

C1	C2	C3
Date / initials	Date / initials	Date / initials
C4	C5	C6
Date / initials	Date / initials	Date / initials

My treatment information

Plan changed? Ask the team to update this card.

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.